

**Laboratory Investigation Report**

Patient Name	: Rajat Srivastava	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 32 Y O M D /M	OP/IP No	: OP/98682
Max ID/Mobile	: 441344/8752005200	Collection Date/Time	: 24/Aug/2021 01:58PM
Lab ID	: 1266082100564	Receiving Date	: 24/Aug/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 24/Aug/2021

Blood Bank**Blood Grouping and RH Factor*, EDTA**

Date	24/Aug/2021 01:58PM	Unit	Bio Ref Interval
Blood Group	A POSITIVE		
Haemagglutination			

Kindly correlate with clinical findings

***** End Of Report *****
Dr. Sangeeta Pathak , DIHBT
Head-Transfusion Med

SIN No: B2B899482, Test Performed at : 1103 - Max Hospital Saket (East Block), 1, 2, Press Enclave Marg, Saket Institutional Area, Saket, New Delhi
Max Lab - A Division of Max Healthcare Institute Ltd.

Booked at: Max Super Specialty Hospital, Saket (West Block), 2nd Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.

8752005200
CIN No.: L72200MH2001PLC322854
The report can be verified by scanning the Q R Code on top of the page
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Conditions of Reporting: 1. The tests are carried out in the lab with the presumption that the specimen belongs to the patient name as identified in the bill/test request form. 2. The test results relate specifically to the sample received in the lab and are presumed to have been generated and transported per specific instructions given by the physicians/laboratory. 3. The reported results are for the information and interpretation by the referring doctor only. 4. Some tests are referred to other laboratories to provide a wider test menu to the customer. 5. Max Healthcare shall in no event be liable for accidental damages loss, or destruction of specimen which is not attributable to any direct and mala fide act or omission of Max Healthcare or its employees. Liability of Max Healthcare for deficiency of services, or other errors and omissions shall be limited to fee paid by the patient for the relevant laboratory services.



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Immunoassay

Prolactin, Serum*

Date	24/Aug/2021 01:58PM	Unit	Bio Ref Interval
Prolactin CLIA	6.56	ng/mL	2.64-13.13

Ref Range

Males :	2.64 - 13.13
Females :	
Premenopausal (<50 years of age):	3.34 - 26.74
Postmenopausal (>50 years of age):	2.74 - 19.64

Interpretation

Increased in prolactin-secreting pituitary tumors, amenorrhea and/or galactorrhea, Chiari-Frommel and Argonz Del Cstillo syndromes, various types of hypothalamic-pituitary disease (e.g. sarcoidosis, granulomatous diseases, craniopharyngioma, metastatic cancer, empty sella syndrome), primary hypothyroidism, anorexia nervosa, polycystic ovary syndrome, renal failure, insulin-induced hypoglycemia, chest wall injury, adrenal insufficiency, and pituitary stalk section surgery
Decreased in pituitary apoplexy (Sheehan's Syndrome)



SIN No: B2B899482, Test Performed at : 896 - Motherland Hospital, Motherland hospital Sec-119

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Immunoassay

TSH,Serum*

Date	24/Aug/2021 02:00PM	Unit	Bio Ref Interval
Thyroid Stimulating Hormone	2.2	µIU/mL	0.34 - 5.6
CLIA			

Interpretation

Parameter	Unit	Premature (28 - 36 Weeks)	Cord Blood (> 37 weeks)	Upto 2 Month	Adult	1st Trimester	2nd Trimester	3rd Trimester
TSH	uIU/ml	0.7 - 27.0	2.3 - 13.2	0.5 - 10	0.38 - 5.33	0.1 - 2.5	0.2 - 3.0	0.3 - 3.0

Increased in primary Hypothyroidism.
Decreased in primary Hyperthyroidism

Note : TSH levels are subject to circadian variation, reaching peak levels between 2 – 4 am and at a minimum between 6 – 10 pm. The variation is of the order of 50% - 206 %, hence time of the day has influence on the measured serum TSH concentrations.

Kindly correlate with clinical findings

*** End Of Report ***



Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



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Serology

Test Name	Result	Unit	Bio Ref Interval
VDRL/RPR*, Serum			
RPR(Syphilis) Slide Flocculation	Non Reactive		

Comment**Interpretation**

1. It is a screening test for syphilis which is useful for following the progression of disease and response to therapy. Rising titers are of immense value in confirming the diagnosis.
2. Biological false positive reactions exhibit low titers and are seen in conditions like Viral fevers, Mycoplasma infection, Chlamydia infection, Malaria, Immunizations, Pregnancy, Autoimmune disorders & past history of Treponemal infection.
3. It is advisable to confirm the diagnosis by tests such as TPHA & FTA-ABS.
4. A definitive clinical diagnosis should not be made by result of a single test only, but should be made by taking clinical history and other laboratory findings in to account.

Kindly correlate with clinical findings

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